

THE SOMATIC RESET SERIES

Somatic Healing for Perimenopause

*Managing Hormonal Anxiety Through the
Body*

Disclaimer

This book is intended for educational and informational purposes only. It is not a substitute for professional medical, psychological, or psychiatric advice, diagnosis, or treatment. Always consult a qualified healthcare provider before making changes to your health routine, especially if you have a pre-existing condition.

The practices described here are general wellness approaches. Results will vary between individuals. The author and publisher accept no liability for outcomes arising from use of this material.

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When Your Nervous System Changes with Your Hormones

Many women in perimenopause — the hormonal transition that typically begins in the late 30s or 40s, sometimes earlier — describe a sudden onset of anxiety that feels qualitatively different from anything they've experienced before. Not situational anxiety tied to a specific worry, but a nervous system agitation that seems to come from nowhere. This chapter explains why, because understanding it makes it less frightening.

Estrogen and the Nervous System

Estrogen has direct effects on the nervous system that go far beyond reproductive function. It modulates neurotransmitter activity — including serotonin, dopamine, and GABA — and has anti-anxiety effects that are broadly protective. It supports the function of the hippocampus, the brain structure involved in memory and in regulating the stress response. It influences the myelination of neural pathways. It has anti-inflammatory properties in the brain.

During perimenopause, estrogen levels fluctuate irregularly — sometimes dramatically — before eventually declining. These fluctuations affect the nervous system directly. The anxiety, mood instability, brain fog, sleep disruption, and emotional reactivity that many women in perimenopause experience are not symptoms of psychological weakness. They are the direct neurological effects of a fluctuating hormone that has been keeping multiple systems in calibration for decades.

This is important because many women in perimenopause present to doctors with anxiety or mood symptoms and are referred to mental health treatment without the hormonal dimension being addressed. Both dimensions may need attention. But the hormonal piece is real and significant.

The Specific Nervous System Presentations of Perimenopause

The anxiety of perimenopause has some characteristic features: it often presents as a physical sensation of internal vibration or agitation, it frequently worsens in the premenstrual phase of an irregular cycle, it is often accompanied by a new sensitivity to stimulation (noise, light, crowds), and it tends to peak in the years of most irregular hormone levels before improving once levels stabilise in postmenopause.

Night sweats are a direct nervous system event — a vasomotor response driven by disrupted thermoregulation that activates the sympathetic system in the middle of the night, producing the cortisol spike that underlies much perimenopausal insomnia. The night sweat itself is uncomfortable; the subsequent inability to return to sleep is a nervous system regulation problem.

Rage — sudden, intense, and often disproportionate — is another perimenopausal nervous system presentation that is rarely discussed with adequate honesty. The loss of estrogen's buffering effect on amygdala reactivity means that the emotional brake is less effective. This is not who you are. It's your nervous system under hormonal pressure.

"What is happening to your nervous system in perimenopause is real, biological, and temporary. It also responds to the same things that have always helped regulate the nervous system — applied with more consistency and more compassion."

Somatic Practices for Hormonal Anxiety

The practices here are specifically adapted for the perimenopausal nervous system — which needs gentler, more consistent approaches and responds less well to high-intensity interventions that add physiological demand.

Breathwork for Hot Flashes and Night Sweats

The physiological sigh — double inhale followed by a slow, complete exhale — is effective for interrupting the vasomotor cascade of a hot flash if used at the first sign of onset. When you notice the initial warmth, double inhale through the nose and release completely through the mouth. Repeat three to five times.

Several small studies have found that paced breathing at roughly six breaths per minute (coherent breathing) reduces the frequency and severity of hot flashes when practiced consistently. The mechanism is likely through HPA axis regulation — the same stress-response pathways that drive vasomotor symptoms are calmed by consistent vagal activation.

For night sweats: the 4-7-8 breath immediately upon waking helps interrupt the sympathetic activation that prolongs the waking episode. Combined with a consistently cool sleep environment and the sleep practices from Book 5 in this series, most women find significant improvement in sleep continuity.

Grounding for Perimenopausal Anxiety Spikes

When perimenopausal anxiety spikes — the sudden, sourceless agitation that can arrive in the middle of an ordinary moment — grounding is the most effective immediate intervention. Feet firmly on the floor, deliberate sensory engagement with the environment, physiological sigh.

Cold water is particularly effective for the perimenopausal nervous system: splashing cold water on the face and wrists provides immediate vasomotor relief while also triggering the calming mammalian dive reflex. Keep a small spray bottle of cold water if you're in situations where this matters.

Yoga Nidra for Hormonal Sleep Disruption

Yoga nidra — often called yogic sleep — is a guided relaxation practice that induces a hypnagogic state

(between waking and sleep) through body scanning and visualisation. Multiple studies have found it effective for insomnia, and it appears particularly well-suited to perimenopausal sleep disruption because it works regardless of whether sleep occurs.

The key benefit: thirty to forty-five minutes of yoga nidra is associated with the equivalent restorative effect of two to four hours of sleep in some research. When genuine sleep is disrupted by night sweats or hormonal anxiety, yoga nidra offers a restorative alternative that doesn't require sleep onset. Recordings are freely available on YouTube and most meditation apps.

"Your nervous system is adjusting to a new hormonal reality. Give it more support during the transition, not less."

Supporting the Whole System

Somatic practices are one dimension of perimenopausal support. This chapter addresses the others.

Nutrition and the Perimenopausal Nervous System

The fluctuating estrogen of perimenopause increases inflammatory signalling and can disrupt blood sugar regulation — both of which directly affect nervous system stability. Dietary approaches that reduce inflammation and stabilise blood sugar are therefore directly relevant to perimenopausal anxiety and mood.

Protein at each meal (particularly leucine-rich sources: eggs, meat, fish, dairy) stabilises blood glucose and supports neurotransmitter synthesis. Omega-3 fatty acids (fatty fish, flaxseed, walnuts) reduce neuroinflammation. Magnesium (leafy greens, nuts, seeds, dark chocolate) supports GABA activity — the brain's natural calming neurotransmitter — and is frequently depleted in perimenopausal women.

Caffeine and alcohol are both more disruptive to the perimenopausal nervous system than they were earlier in life, due to changes in liver metabolism and increased nervous system sensitivity. Neither needs to be eliminated, but their effects are worth monitoring — particularly their impact on sleep and anxiety levels.

Strength Training as Nervous System Support

Resistance training during perimenopause provides benefits that go well beyond muscle maintenance. It improves insulin sensitivity, reduces inflammatory markers, supports bone density (which begins declining with estrogen), and — particularly relevant here — has been shown to reduce anxiety and depression through mechanisms including BDNF (brain-derived neurotrophic factor) release and HPA axis regulation.

Two to three sessions per week of progressive resistance training is the evidence-based recommendation. This doesn't require a gym membership or heavy equipment — bodyweight resistance training (squats, push-ups, hinges, rows) produces the same hormonal and neurological benefits.

When to Talk to a Doctor About HRT

The evidence base for hormone replacement therapy (HRT) has been substantially revised since the Women's Health Initiative study in the early 2000s. Current understanding, based on more recent research and the work of clinicians like Avrum Bluming, Christiane Northrup, and Mary Claire Haver, is that HRT is appropriate for many perimenopausal women — particularly those with significant symptoms — and that the risk profile is more favourable than was previously believed.

This book is not the appropriate place for a detailed HRT discussion, and any decision about hormonal treatment should be made with a qualified healthcare provider who is current on the evidence. The somatic and lifestyle practices in this book are appropriate alongside any treatment approach — they support the nervous system regardless of hormonal management strategy.

"Perimenopause is not the beginning of decline. It is a transition with real challenges and real tools to meet them."

The Other Side

Perimenopause ends. Postmenopause has its own landscape, and it's worth knowing what to expect.

Nervous System Stabilisation in Postmenopause

Once hormones stabilise at their postmenopausal levels — which typically happens within two to three years of the final menstrual period — many women report a significant reduction in the nervous system agitation of perimenopause. The anxiety that felt constant and sourceless often diminishes substantially. Sleep, while still sometimes a challenge, becomes more predictable.

The nervous system practices built during the perimenopausal transition become even more valuable postmenopausally — as prevention and maintenance rather than crisis management. The reduced estrogen environment means the nervous system is slightly more vulnerable to stress activation than it was during the reproductive years, but a consistently practiced regulation toolkit provides significant compensation.

What Stays and What Changes

The intense vasomotor symptoms of perimenopause typically reduce significantly in postmenopause. The sleep disruption usually improves. The anxiety and mood volatility that tracked estrogen fluctuations tends to stabilise.

What doesn't automatically improve without attention: the reduced stress resilience that comes with lower estrogen, the increased importance of physical activity for mood and nervous system health, and the need for proactive management of the conditions — cardiovascular health, bone density, cognitive health — that estrogen was protecting against.

The women who navigate this transition best, in both research and clinical observation, are those who build the practices and relationships that support their nervous system during the difficult years, and carry them forward into the next chapter with the knowledge of their own system that the transition gave them.

*"The nervous system that survived
perimenopause has been through something.
It deserves recognition — and the ongoing
care of someone who now knows it well."*
